June 16, 2025

The Honorable Administrator Mehmet Oz
Centers for Medicare & Medicaid Services
7500 Security Boulevard
Baltimore, MD 21244

Submitted via regulations.gov

Subject: Comments on the Information Health Technology Ecosystem (CMS-0042-NC)

Dear Administrator Oz,

On behalf of the millions of patients and the 175,000+ providers that use Zocdoc across all 50 states, thank you for the opportunity to provide comments regarding digital health products, data access, and interoperability. We appreciate the Administration's focus on unleashing American innovation and modernization in the digital health sector. As a pioneering American healthcare technology organization, we've spent nearly the last two decades bringing market forces to healthcare to address rising costs, diminishing quality, and to increase access across the United States. We've built the nation's leading patient-centered marketplace that allows users to search, compare, and instantly schedule the care they need with the providers of their choice. In doing so, we empower patients, bring efficiency to providers, and save time, money, and resources for the healthcare system in aggregate.

Our goal is to build the connective tissue of healthcare, by stitching together a fragmented industry to work cohesively for the betterment of all Americans. Our unique insights into patient behavior and provider workflows position us to offer valuable feedback on advancing digital health solutions on the pages to come.

Our key recommendations are summarized below:

1) CMS should completely transform the Medicare Compare tool by integrating a bookable directory directly into the site. This is a common sense, cost-saving, fraud-detecting solution that could be deployed quickly and would deliver real results for the American people, leveraging proven technology to improve healthcare.
2) To power true innovation in the digital health sector, we urge the Agencies to take up the safe harbor outlined in the Health ACCESS Act.
3) To achieve truly meaningful interoperability for the benefit of patients, providers, and the health technology ecosystem, we urge the Agencies to expand the scope of information blocking rules to include administrative data.

Patients and Caregivers

Patient needs (PC-1 through PC-7): Empowering Patients Through Seamless Care Navigation

Imagine a healthcare system where finding and booking an appointment is as straightforward as reserving a restaurant table or booking a flight. This vision is at the heart of Zocdoc's work, and it's precisely what a national, bookable Medicare directory could achieve for America's seniors. The current landscape for Medicare beneficiaries is often characterized by frustration, friction, and confusion. Our own secret shopper analysis of Medicare Compare revealed a dismal 3.6% success rate in booking a new patient appointment, meaning the current tool fails seniors more than 96% of the time.[1] This fragmentation and inaccuracy in provider directories, often referred to as "ghost networks," leads to significant barriers to care and unnecessary emergency room visits.

To truly empower patients, a modern, bookable provider directory must enable the following key functionalities, all of which Zocdoc is proud to support today:

- **Instant, Online Booking:** Patients and caregivers need to be able to see real-time provider availability and book appointments directly, eliminating time-consuming phone calls.
- **Accurate Provider Information:** Directories should provide users with access to reliable, up-to-date details like name, specialty, location, contact information, and board certifications.
- **In-Network Clarity:** Users must be able to easily identify providers who accept their specific insurance plans in order to effectively manage their costs.
- **Informed Choice:** Patients must be empowered to research options based on what matters most to them—be it insurance, reviews, location, or availability.

We are encouraged by CMS's recent announcement about the intention to build a national healthcare directory, starting with provider information. We feel strongly that the directory will only be useful if scheduling information is mandated so an appointment can actually be made seamlessly on the site. This will accelerate access to care. On Zocdoc, because we can surface last minute openings in a providers' calendar in real time and make them instantly bookable to patients, a typical appointment takes place within 24-72 hours, significantly faster than the national average wait times, which can stretch to 31 days or more.[2] This speed is vital as delayed care often leads to worsening conditions, missed diagnoses, and increased costs.

Beyond online booking, imagine a dedicated personal assistant for healthcare administration, powered by AI. This assistant would not only book appointments instantly by leveraging real-time

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[1] We recently conducted a secret-shopper analysis of DC, Miami, Philadelphia, and Dallas with the goal of booking an annual well visit via Medicare Compare. We called 136 practices in search of a primary care provider with availability to see a new patient. Only 5 of them offered an appointment. Notably, 3 out of 5 providers were already participating in the Zocdoc marketplace.

[2] AMN Healthcare - 2025 Physician Appointment Wait Times

availability—even last-minute cancellations—but also help navigate complex insurance plans, verify provider credentials, refill prescriptions, recommend referrals, and offer convenient, after-hours booking options. The impulse to book care often strikes outside of traditional office hours; over a third of Zocdoc appointments are booked between 5 PM and 9 AM, and 17% on weekends. This "anytime booking" capability is crucial for ensuring timely access to care and reducing costly emergency room visits, as nearly one in five Zocdoc users who booked a same-day appointment stated they might have gone to the ER otherwise.

This personal assistant for healthcare administration is precisely what we have built. We are proud to have recently launched Zo by Zocdoc, an AI Phone Assistant that eases one of the most frustrating bottlenecks for both patients and healthcare organizations: the phone. Zo engages like a human and manages appointments like an expert, handling unlimited inbound scheduling calls to increase patient access and retention for healthcare organizations. On the patient end, Zo vanquishes hold times and instantly schedules appointments 24/7 using natural, conversational language.

Zocdoc is an off-the-shelf solution that is already beloved by patients and works at scale. We stand ready to bring nearly 20 years of healthcare scheduling expertise and industry leadership to America's seniors, and are open to working in partnership with any organization that shares our vision. Zocdoc can leverage our existing scheduling platform within months to build a national, bookable Medicare provider directory, improving Medicare Compare within months. This directory can be paired with a 24/7 scheduling hotline within 1-800-Medicare, powered by Zo. Collectively, our technology would greatly improve efficiency and reduce costs for CMS by providing real, upstream appointment data back to CMS on booking behavior to detect and root out fraud, waste, and abuse.

Data Access and Integration (PC-8 through PC-12): The Challenge of Access and Interoperability

While the vision of seamless care is clear, its realization faces significant hurdles. Patients currently struggle with fragmented health information, scattered across disjointed provider systems and patient logins with limited interoperability. To date, information blocking rules and regulations are asymmetrical, overindexing on giving *providers* what they need but not doing nearly enough to let *patients* make informed choices themselves.

Unlike clinical data, which has seen some standardization, administrative data (e.g. calendar, appointment time slot, etc.) is largely unregulated, leading to inconsistent formats and a lack of open APIs. This grey area reinforces the "walled garden" approach of some Electronic Health Record (EHR) vendors, who may restrict access to crucial administrative data like provider scheduling rules and availability, turning it into a feature that undermines and blocks true interoperability.

We appreciate the Agency's specific inquiry into what would be very valuable but may be very hard to

achieve. We believe a truly valuable solution would be an EHR-agnostic bookable directory, as described above, with up-to-date availability and quality metrics, though it remains elusive without intervention. However, to deliver on this promise of a comprehensive, bookable Medicare directory—and a more seamless and connected healthcare experience, more broadly—administrative tasks must be explicitly included in information blocking rules, so digital health innovators can work on behalf of the patient's interests.

To solve this, we urge ONC/ASTP to consider opening up the information blocking rules to include data related to administrative tasks and functions.

Digital Identity (PC-13 through PC-14): Portability Needed for True Accountability

While the technical challenge of digital identity credentials not being easily transferable is significant, the true obstacle to patient adoption is that they offer insufficient practical value to patients. Patients have no, or little, visibility into what that identity can power, how to use it, or what is being communicated about them under the hood. Patient Access mandates, even when properly implemented by Health IT, provide little transparency or utility to patients, and this is largely because these access requests are as fragmented as the Health IT ecosystem itself.

Zocdoc is heartened by the Administration's efforts to encourage aggregation of provider information, and this theme should be extended to Patient Access as well. Because Zocdoc works across a vast and diversified network of providers, scheduling software and payers, we already help patients achieve greater visibility into their care journey than any one EHR or payor provides. We intend to continue in this effort—encouraging patients to take advantage of their rights over their own patient data, aggregate it, and give them the information they need to make informed decisions about their own care.

Providers

Digital Health Apps (PR-1 through PR-4): Cultivating Provider Engagement with Digital Health

To encourage healthcare providers, particularly those in rural areas, to more effectively leverage approved digital health products for their patients, CMS and its partners must address several significant obstacles. The current regulatory ambiguity surrounding the Anti-Kickback Statute (AKS)[3] can unfortunately stifle innovation and create uncertainty for providers seeking to partner with digital health platforms. This lack of clarity often impedes the adoption of common sense, best-in-class technology that could otherwise improve healthcare access and reduce costs.

[3] 42 U.S. Code § 1320a-7b

The AKS governs improper payments for referrals and offers an important guardrail to prevent fraud and abuse. However, the law was written in 1972—decades before the advent of the Internet or today's innovative digital health technology—and is written in such an overly broad manner that it can implicate innumerable legitimate business practices. As a result, it outmoded and it has been necessary, over the half-century since its enactment for Congress, to add dozens of safe harbors and for the HHS-OIG to issue hundreds of Advisory Opinions; these can serve to clarify that typical business practices that encourage efficiency do not present risk of fraud and abuse are permissible under Federal law.

Health information service providers (including online appointment booking platforms) perform one such business practice by leveraging technology to provide a crucial link between patients and providers and ensure seamless access to healthcare. To participate in an online booking platform, providers typically provide compensation to the platform. However, under current law, there is no distinction between illicit referral practices that incentivize fraud and waste in contrast to legitimate and useful information, marketing and scheduling services that simply reduce federally funded beneficiaries' barriers to accessing necessary and appropriate care.

As we've written in other recent regulatory inquiries, we believe a health information services safe harbor would provide clear guidelines to modernize this decades-old regulation and remove the ambiguity that exists for American healthcare innovators, and the patients and providers they serve. This would allow patients and providers to use digital tools to make healthcare more accessible and efficient, and in doing so help providers throughout hospitals, health systems, community clinics, and independent practices operate more efficiently and improve access to care.

The Health ACCESS Alliance represents a growing list of 30+ organizations, which include hospitals, health systems, FQHCs, health tech innovators, patient advocacy orgs, trade associations and more. Zocdoc has joined the Health ACCESS Alliance to advocate for common sense modernization through their support for the Health ACCESS (Accelerating Consumer's Care by Expediting Self-Scheduling) Act,[4] which is a bipartisan, bicameral bill that would create such a safe harbor. This would allow health information services to help Americans, including federally funded beneficiaries, find care more easily, without unnecessary legal uncertainty for the providers that utilize these services.

Separately, providers are equally burdened by the lack of protection against information blocking for administrative functions as discussed above. When EHRs present integration challenges like not easily connecting with third-party digital health products—at providers' direction—for crucial administrative functions like scheduling, providers' real-time availability may not be able to be displayed to patients seeking to book an appointment. This technical hurdle contributes to the administrative burden

[4] S. 1140 and House introduction to follow.

providers face, as the effort required to integrate new technologies and maintain multiple digital systems can be a significant deterrent.

To foster a more integrated and efficient digital health ecosystem, changes are needed in current standards and policies, particularly regarding access to administrative workflows. There is a pressing need for explicit regulatory clarity in the information blocking rules, to prevent EHRs from "gating" essential administrative data. Restricting access to this data directly impacts patient access to care and hinder America's digital health innovators from improving Americans' access to, cost and quality of care.

While existing FHIR standards are generally sufficient for scheduling functionalities, a critical area for improvement lies in standardizing patient-facing terminology. The most salient example of non-standard scheduling criteria centers on something as simple as "visit reason" / "visit type." Not only is there nothing standardized here, but the very thing these represent are also not standardized. In our experience, most EHRs are only flexible enough to set slots as available for 1) New Patients, 2) Annual Physical, or 3) Medicare. But even those vary widely in meaning — some function merely as labels akin to a sticky note on a desktop monitor, and some are enforceable scheduling limitations within the EHR. In the latter case, a slot will not surface if the patient is an established patient, isn't looking for an annual physical, or has commercial insurance. There's a high level of pre-coordination that Zocdoc undertakes to navigate these complexities on the back-end so patients do not have to on the front-end, but SMART Scheduling Links seeks to avoid any pre-coordination between systems. Without pre-coordination, patients either drop out of the experience when they attempt to violate a rule encoded in the EHR (unbeknownst to them), or they seemingly successfully schedule only to get a phone call about scheduling the wrong time — or even worse, show up only to learn that they are unable to receive care. This would be an unacceptable consumer experience in any other industry and should be in healthcare as well.

This is essential for informing the codesets necessary for accurate patient-to-provider and appointment slot matching, including the standardization of visit reasons and insurance plan codesets. These advancements would pave the way for a truly seamless and patient-centric healthcare experience, while making providers' schedules and throughput more efficient.

Information Blocking (PR-12 through PR-14): Recognizing the Importance of Administrative Data

To encourage providers to submit information blocking complaints to ASTP/ONC's Information Blocking Portal, and thus encourage compliance, CMS should expand the scope of information blocking rules to explicitly include administrative data. Currently, administrative data is not specifically addressed by

these rules. If administrative data were included, providers would have a greater incentive to file complaints whenever a third-party application they've selected to perform an important task for their organization and/or their patients is denied access by an EHR system.

The impact of such a change would be highly positive, significantly advancing data exchange. It would dismantle barriers erected by EHRs that currently silo critical administrative information. This, in turn, would foster increased competition among digital health innovators while alleviating the administrative burden on providers who frequently grapple with fragmented systems.

Technology Vendors, Data Providers, and Networks

Ecosystem (TD-1 through TD-2): Case study on Zocdoc API

To effectively stimulate developer interest in continuing to build digital health products for Medicare beneficiaries and their caregivers, CMS must focus on enhancing data accessibility and expanding its API offerings.

While the Blue Button 2.0 API provides valuable patient demographic, Medicare coverage, and claims data, developers need access to additional, high-value data sources to create truly impactful digital health products. For Zocdoc, the most valuable additional data is real-time provider availability and scheduling data, which we have built a clearinghouse for over the past two decades. We have made this data available as an API that developers can build on top of to allow for instant appointment booking, a core functionality for improving access to care and eliminating "ghost networks." Our early use cases here have been to power scheduling for Community Care Network bookings for the VA as part of the MISSION Act, and powering Provider Directories for large payors including Blue Shield of California. Digital bookability has already cut wait times for veterans by 65% through the VA and improved care coordinator efficiency by nearly 3x.[5]

Beyond scheduling, comprehensive and standardized provider directory information, including NPI, specialty, accurate contact details, and board certifications, are all vital elements on the provider side. Information on insurance plan acceptance and detailed network participation is highly valuable for patients navigating their options.

Currently, significant obstacles prevent developers from accessing these crucial data sources. The primary challenge is the lack of standardization across various provider systems and EHRs, leading to fragmented and inconsistent data formats. Many EHRs and providers do not offer open APIs for

[5] U.S. Senate Committee on Veteran's Affairs, "Sens. Moran, Fischer, Boozman, Budd Introduce Legislation to Improve Community Care Scheduling for Veterans."

administrative data, again, creating "walled gardens" that hinder real interoperability. This is compounded by the fact that administrative data is not adequately covered by current information blocking rules, providing little incentive for data sharing.

To unleash innovation and better support patients and providers, CMS and ONC/ASTP should consider including policies that specifically mandate the availability of APIs for administrative data. This would push for greater interoperability in areas like appointment management, intake forms, and the real-time viewing of providers' schedules. By treating administrative data similar to how clinical data is handled under information blocking rules, CMS can create a powerful incentive for EHRs to open up access, fostering a more competitive and innovative digital health ecosystem. This strategic expansion of accessible data and API mandates would empower developers to build solutions that truly simplify healthcare navigation and access for Medicare beneficiaries.

Technical Standards and Certification (TD-4 through TD-11): Beyond interoperability

To cultivate a truly accessible, not just interoperable, healthcare ecosystem, CMS must expand its promotion of open, standards-based, publicly available APIs to include administrative functionality. This shift is critical for unlocking innovation and access.

A nationwide provider directory of FHIR endpoints will only be successful if it benefits patients in the way they need most—finding and accessing care, and highlighting real providers that can meet their needs. Such a directory needs to improve access to health information for patients, providers, and payors by providing a single, reliable source of truth for provider data and enabling real-time appointment booking. This would eliminate the pervasive issue of "ghost networks" and significantly reduce patient wait times.

CMS is uniquely positioned to promote such a directory, ideally built upon existing infrastructure but greatly enhanced with bookable capabilities. Crucially, users—patients and caregivers—should not bear any cost for accessing this essential information. Zocdoc stands ready to offer its nearly two decades of unmatched scheduling expertise and proven-at-scale booking platform to CMS to help power scheduling for this national Medicare provider directory.

The USCDI has undeniably improved interoperability, but its limitations, particularly concerning the breadth of data elements, must be addressed. It often does not contain the full extent of data elements necessary for complete patient management. Adding more data elements, particularly in administrative areas, would add significant value. While such additions might create scoping challenges, these can be managed through iterative development and close collaboration with stakeholders. Given advancements in language models, considering a non-proprietary but less

structured format might improve data coverage, even if it requires more processing by the receiver, offering a flexible pathway to richer data exchange.

Compliance (TD-18 through TD-19): Examples of information blocking and looking towards a more transparent future for prices

Zocdoc has built more than 175 calendar integrations with EHRs and practice management systems, and while the vast majority are partners in our efforts, some have proven to only comply with the letter and not the spirit of the interoperability frameworks that exist today. This is where the lack of explicit oversight of administrative data creates barriers to care for patients. As a technology vendor in the healthcare space, we have unfortunately encountered various practices that may constitute information blocking, hindering our ability to provide seamless services and improve patient access.

- **Non-responsiveness and Deliberate Obstruction of Administrative Data Access:** A primary example is the difficulty in accessing real-time provider availability and scheduling information from certain EHR systems or provider organizations. Despite the clear benefits for patients seeking appointments, and the fact that we are acting on our shared provider clients' direction to integrate with the system they've purchased and run their practice on, some entities are non-responsive to requests for API access or deliberately create technical barriers to integration, effectively "gating" this crucial administrative data. This means that while patients expect to book online for healthcare services like they do for other industries, the necessary data is withheld or made inaccessible.
- **Limiting API Access for Competitive Reasons:** Some EHR vendors may limit access to their APIs for third-party applications, especially for administrative workflows, because these functions are often where they desire to invest product effort to differentiate themselves from competitors. This still creates a situation where the vendor's commercial interests supersede the broader goal of patient access and efficient data exchange. We are beginning to see increased litigation in this area, which shows that further regulatory intervention may be necessary.

ASTP/ONC and CMS could implement several additional policies to further discourage information blocking practices as follows:

- **Expand Information Blocking Rules to Include Administrative Data:** Critically, the current information blocking rules do not explicitly contemplate administrative data. These rules should be broadened to encompass data such as real-time appointment availability, scheduling information and rulesets, and comprehensive provider directory details. This expansion would directly address the siloing of crucial information by EHRs and remove existing barriers to data exchange.

- **Establish Clearer API Standards and Mandates for Administrative Workflows:** Beyond general information blocking, CMS should actively promote and potentially mandate the use of open, standards-based APIs for administrative functions like scheduling, appointment management, patient intake forms, and prices. This would reduce the reliance on proprietary systems and force greater interoperability.

Despite information blocking rules covering diverse entities and situations, the lack of clear penalties for blocking specific data categories poses a challenge. Notably, the absence of direct disincentives for blocking administrative data—data crucial for patient access and optimal provider use—is a significant deficiency. Although information blocking is broadly defined, concrete methods to penalize those obstructing patient access to appointment information are underdeveloped. This weakness enables the spread of "ghost networks," where provider directories appear extensive but patients face significant hurdles in scheduling appointments.

We are pleased to see this Administration's focus on getting price transparency in healthcare right. Today, there are significant hurdles for patients, providers, and payors alike. The information provided is often insufficient making it difficult for patients to understand their true out-of-pocket expenses. The format in which this limited information is presented is frequently inconsistent and difficult to navigate, while delivery methods are often far from user-friendly or seamlessly integrated into existing patient workflows. The collective lack of clear and actionable price data places an additional, unnecessary burden on patients as they attempt to navigate the complexities of healthcare costs.

Functional price transparency holds the potential to revolutionize several key healthcare workflows. In every other sector consumers benefit from transparent prices that they understand before the transaction occurs. The consumer experience of shopping for healthcare services stands to benefit immensely, allowing patients to view accurate cost estimates before committing to an appointment, thus empowering them to make informed decisions that balance both clinical necessity and financial considerations. In referral management, readily available price information could guide patients toward more cost-effective, in-network specialist options. For care planning related to chronic conditions, clear understanding of associated costs would enable patients and providers to better plan for ongoing treatment.

The most valuable improvements to price transparency would yield distinct benefits for all stakeholders. For patients, access to binding, upfront cost estimates for specific services would be invaluable, allowing them to compare options effectively and avoid the unwelcome surprise of unexpected bills. Providers would gain streamlined processes for delivering price information to patients and reducing administrative burden associated with billing inquiries and disputes. Payors, including CMS, would see an enhanced ability to guide patients toward more cost-effective care settings, ultimately leading to a reduction in wasteful spending across the system.

To truly motivate the development of robust price transparency solutions, several strategic actions are necessary. Mandating clear, standardized APIs for price transparency data is paramount, as this would empower technology vendors to build innovative tools that integrate pricing information directly into patient-facing applications. Additionally, incentivizing providers and payors for the submission of accurate and timely price data—perhaps by linking it to quality metrics or reimbursement—could significantly motivate compliance.

Zocdoc stands ready to partner with CMS to bring generational change to the health IT ecosystem

We appreciate—and share—CMS's ambitious goals outlined within this RFI: to achieve IT modernization that leads to high-quality, affordable, and sustainable care, reduces fraud, waste, and abuse, and enables real-time information access for providers and patients alike. Zocdoc stands ready to offer our nearly two decades of expertise, as well as our partnership as you embark on this generational opportunity. Unlocking this is key to delivering a more efficient, more accessible, more affordable, high quality healthcare system our nation deserves.

We believe the three key themes outlined throughout our thoughts above and consolidated below, will be transformative to the digital health sector, help root out upstream fraud, waste, and abuse, and unleash innovation in this space:

1) We can completely transform the usefulness of the Medicare Compare tool by integrating our booking functionality directly into the site. This is a common sense, cost-saving, fraud-detecting solution that could be deployed quickly to deliver real results for the American people, leveraging proven technology to improve healthcare.
2) To power true innovation in the digital health sector, we urge HHS-OIG to take up the safe harbor outlined in the Health ACCESS Act.
3) To achieve truly meaningful interoperability for the benefit of patients, providers, and the health technology ecosystem, we urge the Agencies to expand the scope of information blocking rules to include administrative data.

Thank you, again, for the opportunity to provide comments on this matter. We would be delighted to expand on our comments or provide any additional information that might be helpful.

Sincerely,

Oliver Kharraz, M.D.
Zocdoc Founder and CEO